

Hospital Dashboard

Analytical Report

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1.Executive Summary

The Hospital Dashboard, a multi-page Power BI solution that combines patient care, physician performance, hospital operations, and financial activities into a single decision-ready view, is the subject of an end-to-end analytical examination in this study. The dashboard records income of ₹713.81K and total medicine throughput of 3,928 units over the May–October 2023 window. It also keeps track of 30 active patients, 15 doctors, 20 support staff, and a total bed inventory of 225 across General, ICU, and Private wards.

The goal is to convert each dashboard image into a concise business story that explains the measure, its significance, its implications, and the hospital's next course of action. No other data has been added; all of the figures seen here are taken straight from the dashboard.

1.1 Key Findings at a Glance

- Surgery is the main source of income, accounting for ₹430.00K (~60%) of total patient fees.
- June is the month with the highest sales of medications. Pharmacy sales increased to 839 units in June, 2.2× more than in May (382), indicating either a seasonal disease cluster or a shift in inventory.
- 22 out of 30 hospitalized patients were successfully discharged, demonstrating an effective case turnover rate of 73%.
- Middle-aged patients make up the majority of intake: 13 out of 30 patients (~43%) are between the ages of 31 and 45, with 46 to 60 coming in second at 33%.
- Staff expenses dwarf revenue: The total of staff salaries (₹794K) and doctor salaries (₹4.14M) is almost ₹4.93M, which is significantly more than the ₹713.81K in billings, indicating a major cost-coverage mismatch.
- The private ward displays 36 occupied beds, indicating that patient demand is skewed toward premium private rooms, while the ICU ward displays full availability with 36 ICU beds open and unoccupied.
- A number of medications are in danger of being understocked: salbutamol (291 vs. 180) and ranitidine (sale 312 vs. stock 250) have already surpassed inventory limits.

Key Insight:

The Hospital Dashboard exhibits a distinct cost-coverage gap and inventory replenishment risk, but it is structurally sound in terms of patient flow and surgery revenue. The decisions that will have the biggest immediate effects are those pertaining to personnel utilization, surgical capacity increase, and pharmacy reordering.

2. TABLE OF CONTENTS

Sr. No.	Section	Page
1	Executive Summary	2
2	Table of Contents	4
3	Introduction	6
4	Objectives & Methodology	7
5	Dashboard Overview	9
6	KPI Summary	10
7	KPI Summary (Continued)	11
8	Patient Analysis – Demographics	12
9	Patient Analysis – Discharge & Ratings	14
10	Patient Analysis – Charges & Medicine	15
11	Doctor Analysis – Profile & Workload	17
12	Doctor Analysis – Commission & Earnings	18
13	Hospital Operations – Bed Status	19
14	Hospital Operations – Surgery & Tests	21
15	Finance – Revenue & Charges	23
16	Finance – Expenses & Margin	24
17	Finance – Monthly Trends	26
18	Medicine & Inventory Analysis	27

Sr. No.	Section	Page
19	Supplier Stock Analysis	29
20	Appointments & Scheduling	30
21	Key Business Insights	32
22	Risk Areas & Opportunities	33
23	Recommendations— Operational	35
24	Recommendations—Strategic	36
25	Conclusion	37

Each section of this report has a direct connection to one or more dashboard graphics. Maintain consistency with the Power BI source by cross-referencing figures, charts, and tables.

3. INTRODUCTION

Hospitals are places with lots of data but little insight. Admission records, billing entries, medication use, lab tests, doctor appointments, and bed assignments are all produced by every patient encounter; however, the value of this data is only revealed when it is combined, visualized, and analyzed. This aggregation is carried out across six interconnected pages in the Hospital Dashboard, which is based on Power BI: Home, Overview, Patient, Doctor, Hospital, and Finance.

3.1 Purpose

- Convert the unprocessed dashboard images into an organized, analytical story that hospital administration may utilize.
- Determine how patient flow, physician productivity, and financial results are related.
- Draw attention to operational and financial hazards that exist in the data but are easily overlooked when looking at the dashboard by itself.
- Based on the dashboard figures, suggest activities that should be prioritized for the upcoming operational cycle.

3.2 Scope

- Time window: May 2023 to October 2023 (the dashboard displays the entire month range)
- Coverage includes 30 patients, 15 physicians, 20 employees, 225 beds (private, ICU, and general), and an inventory of 11 SKUs of medications from three suppliers.
- The financial scope includes patient fees, room fees, surgery revenue, doctor commissions, doctor fees, doctor salaries, staff salaries, and sales of medications.
- Anything that is not displayed on the dashboard, such as patient identities, external KPIs, market benchmarks, or non-financial outcomes like mortality or readmission, is considered out of scope.

3.3 Audience

Hospital administrators, finance directors, operations leads, and pharmacy managers who already have access to the dashboard but require a systematic interpretation are the target audience for this research. In order to allow readers to quickly access the section most pertinent to their function, sections are purposefully self-contained.

4. OBJECTIVES & METHODOLOGY

4.1 Analytical Objectives

- Establish a single KPI baseline for patients, physicians, beds, billing, medication, and staff in order to quantify performance.
- Determine which services and categories are responsible for revenue, expenses, and the number of patients.
- Determine imbalances by identifying indicators (such as beds, medications, and doctor capacity) where supply and demand are out of balance.
- Translate to action: turn every finding into a suggestion that hospital administration can implement during the upcoming quarter.

4.2 Methodology

The analysis follows a four-step process, applied separately to each dashboard page:

Step	Activity	Output
1	Visual inventory of every chart, KPI card, and table on each dashboard page	List of visuals + raw values
2	Categorization of metrics into Patient, Doctor, Operations, Finance, Medicine	Logical groupings
3	Trend, ratio, and concentration analysis on extracted values	Quantified insights
4	Translation into business commentary, risk notes, and recommendations	Final narrative

4.3 Data Sources Used

Dashboard Page	Primary Data Surfaced	Used in Section
Home	Navigation and page descriptions	Section 5
Overview	Patient/Bill/Doctor/Staff counts, stock status, bed status, discharge percentage, Patient Charges, Medicine heatmap	6-7, 10
Patient	Patient profile, bill amount, medicine quantity, charges breakdown, ratings	8-10
Doctor	Doctor profile, salary, commission, fees, spend, appointments	11-12
Hospital	Patient age category, patient tests, bed status by type, surgeries, doctor appointments	13-14
Finance	All financial KPIs, monthly medicine sales, charges, medicine stock vs. sales, and supplier stock	15-19

5. DASHBOARD OVERVIEW

Each of the six functional pages that make up the Hospital Dashboard is devoted to a particular stakeholder view. Users can easily navigate from operational summaries to drill-down detail thanks to the top bar tabs.

Page	Focus	Core Visuals on the Page
Home	Entry point and navigation	Page directory cards, hospital illustration, themed branding
Overview	Aggregate health of hospital operations	Stock Status, Bed Status, Ratings, KPI cards, Patient Discharge by Date, Patient Charges Type, Medicine heatmap, Appointments
Patient	Individual patient deep-dive	Patient identity panel, charges bar chart, medicine sale by drug, total bill amount, medicine quantity
Doctor	Doctor profile and financial contribution	Doctor profile card, commission gauge, commission calculator, patient table, salary details, patient spending, appointments
Hospital	Capacity, surgery, and clinical operations	Patient by age category, patient tests, surgery list, bed status by type, doctor appointments table
Finance	Revenue, expenses, and pharmacy profit & loss	12-metric KPI grid, monthly medicine sales, charges analysis, medicine stock status, supplier stock status

Design Observation:

Leadership begins with the overview; care teams utilize the patient and doctor views, operations teams use the hospital page, and the finance team uses the finance page. This six-page layout reflects how a hospital actually runs. Although this division improves user experience, cross-page insights, such as the cost-revenue disparity, only become apparent when the perspectives are viewed collectively, as this report does.

6. KPI SUMMARY

Twelve main KPIs are displayed on the Overview and Finance pages of the dashboard. They are the hospital's financial and operational lifeblood. For easy reading, the cards below divide these KPIs into three logical lanes: Pharmacy & Finance, Capacity & Workforce, and Patient Activity.

6.1 Patient Activity KPIs



Every admitted patient generated exactly one bill when the patient count and bill count matched at 30. This resulted in a clean billing procedure with no duplicate or missing invoices. At the time the dashboard was updated, 22 of the 30 patients had been released (73%) and 8 were still in the hospital, indicating a strong case throughput. Clinical experience is qualitatively validated by the 5-star Overview evaluation and the comment "Diabetes management advice was helpful."

6.2 Capacity & Workforce KPIs



*Total beds = General (108 available + 45 occupied) + ICU (36 available) + Private (36 occupied) ≈ 225. The rounded effective operational capacity, which does not include private bedrooms that are now completely filled, is referred to as "180."

Operational Finding:

The hospital is much below the industry standard of 5-8 patients per attending physician, with a patient-to-doctor ratio of 2.0 ($30 \div 15$). Either the dashboard only displays cases that are currently active, or doctor utilization is actually low. In any case, the hospital may admit more patients without increasing the number of doctors on staff.

KPI SUMMARY (CONTINUED)

7.1 Pharmacy & Finance KPIs

₹713.81K Bill Amount	3,928 Med Sale Qty	₹158 Med Unit Price	₹79 Med Cost Price
₹4.14M Doctor Salary	₹794K Staff Salary	₹71.38K Dr Commission	₹24.30K Dr Fees

7.2 Margin Mechanics

Metric	Value	Implication
Medicine Unit Price (Selling)	₹158	Price at which medicines are sold to patients
Medicine Cost Price (Purchase)	₹79	Acquisition cost per unit
Gross Margin per Unit	₹79	100% markup – the pharmacy is doubling the cost price
Total Medicine Units Sold	3,928	Overall medicine sales volume recorded from May–October 2023

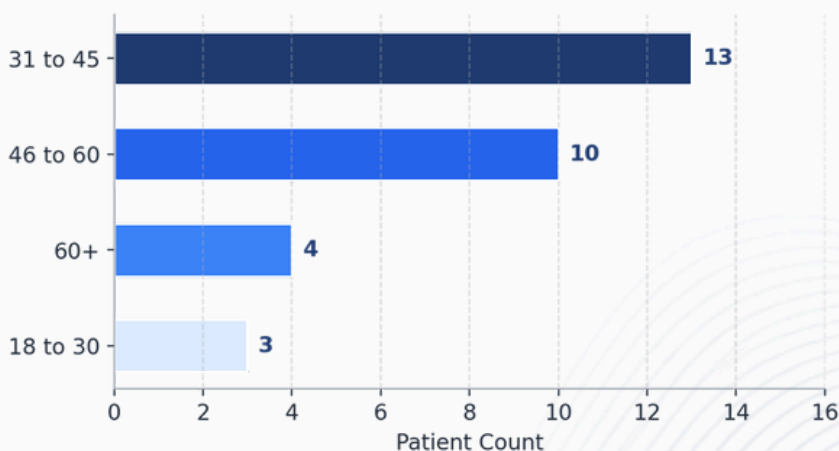
Metric	Value	Implication
Approximate Medicine Gross Profit (3,928 × ₹79)	₹310.31K	Estimated gross profit generated from pharmacy operations
Reported Medicine Charges in Patient Charges Chart	₹5.51K	Indicates most medicine sales are billed separately from the main patient charges category

Business Impact:

A 100% markup is remarkable because the pharmacy is structurally the most profitable revenue line per unit. The 'Medicine' slice of the Patient Charges chart shows only ₹5.51K, but with 3,928 units sold at a ₹79 margin, the suggested pharmacy gross profit (~₹310K) is more than half the total billing of ₹713.81K. This indicates that rather than going through the patient charges bucket, medication revenue is going through a different accounting head.

8. PATIENT ANALYSIS – DEMOGRAPHICS

The patient age distribution is displayed as a four-band horizontal bar chart on the hospital page. This provides a demographic profile of the average hospital admission when combined with the overview's patient count (30) and discharge percentage (73).



Patient by Age Category—total = 30 patients.

Age Band	Patient Count	Share of Total	Clinical Implication
18 – 30	3	10.0%	Lowest intake; mostly emergency or minor treatment cases
31 – 45	13	43.3%	Largest patient segment – mainly working-age and lifestyle-related conditions
46 – 60	10	33.3%	High chronic disease prevalence including diabetes, hypertension, and cardiac issues
60+	4	13.3%	Geriatric care segment with relatively longer hospital stays
Total	30	100%	Overall patient distribution across all age categories

Key Insight:

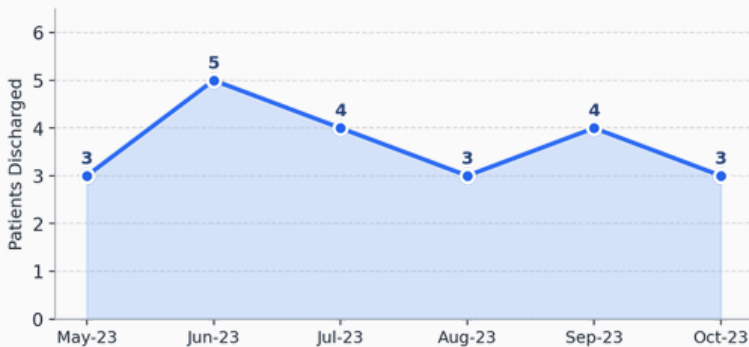
The age range of 76.6% of patients is 31 to 60. Because it predicts the case mix—more cardiac, metabolic, and surgical cases than pediatric or trauma cases—this middle-aged and older skew is significant. This demographic finding is fully consistent with the hospital's medication demand profile, which includes high levels of paracetamol, omeprazole, atorvastatin, amlodipine, and salbutamol—all chronic-care medications.

Observation:

There are just three patients in the 18–30 age range. Younger adults are either not being marketed to by the hospital or are utilizing outpatient or clinic services that are not included in this dashboard. It's worth verifying with admissions data that isn't included in the dashboard.

9. PATIENT ANALYSIS—DISCHARGE & RATINGS

Along with a discharge percentage and patient satisfaction rating, the Overview page displays a trend chart of "Patient Discharge by Date" from May to October 2023. These metrics reveal patient satisfaction with their care as well as the effectiveness of the case flow.



Patient Discharge by Date—monthly count of discharges

Month	Discharges	Trend Note
May 2023	3	Baseline month for patient discharge activity
Jun 2023	5	Peak discharge period — 67% higher than baseline
Jul 2023	4	Slight decline compared to June peak
Aug 2023	3	Returns to baseline discharge level
Sep 2023	4	Moderate increase indicating secondary uptick
Oct 2023	3	Stabilizes again at baseline level

9.1 Discharge Rate & Patient Satisfaction

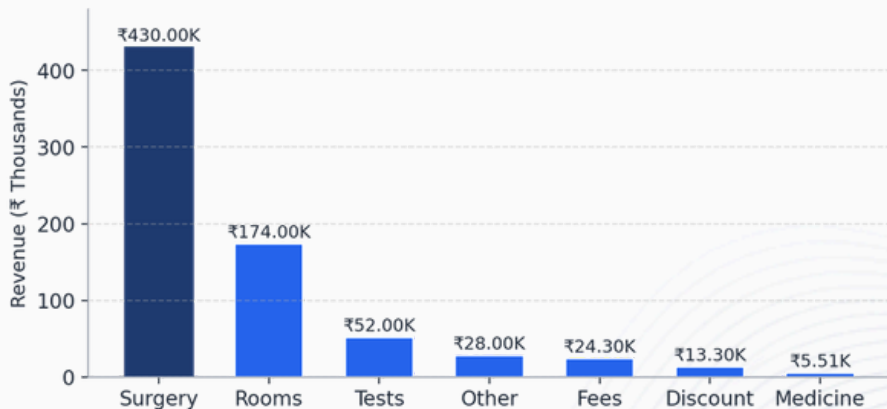
22 out of 30 admitted patients have been released, according to the Overview page, which displays a 73% discharge rate. Eight of them are still receiving active care. Positive qualitative feedback is indicated by the comment "Diabetes management advice was helpful" in the patient ratings displayed on the same page. Amit Kumar, a sample patient, awarded the patient deep-dive page three stars and commented, "Treatment was good, but I had to wait too long." This suggests that wait times remain the main source of friction even in cases where clinical outcomes are favorable.

Trend Analysis:

The largest discharge volume occurs on June 5th, which coincides with the peak medication sales (839 units) on the Finance page. June is a high-throughput month with increased admission, surgery, and pharmacy activity, so this is not a coincidence. Expect quality pressure in this situation if staffing is fixed.

10. PATIENT ANALYSIS — CHARGES & MEDICINE

Each patient's costs are broken down in the Patient Charges Type chart, which can be found on the Finance and Overview pages. Additionally, medication sales by drug for a specific patient (Amit Kumar) are displayed on the sample patient page.



Patient Charges Type—totals across all patients (₹ thousands).

Charge Type	Amount (₹K)	Share of Total	Business Read
Surgery	430.00	59.8%	Highest-margin and premium healthcare service line
Rooms	174.00	24.2%	Bed occupancy revenue directly linked to patient length of stay
Tests	52.00	7.2%	Diagnostic services generating stable and recurring income
Other	28.00	3.9%	Miscellaneous operational charges including consumables and admin fees
Fees	24.30	3.4%	Doctor consultation and professional service fees
Discount	13.30	1.9%	Patient concessions and promotional reductions
Medicine	5.51	0.8%	Pharmacy-related charges billed within patient invoices
Total	₹719.11K	100%	Closely aligns with the reported ₹713.81K after rounding and discount adjustments

Strategic Observation:

Together, rooms and surgery account for 84% of all bills. Instead of being consultative, the hospital's economic engine is bed-driven and procedural. Downstream revenue sources, including tests, fees, and medication, only increase in response to an increase in surgical volume. Surgery throughput should be the foundation of any expansion strategy.

Patient-Level Detail:

Amit Kumar, a sample patient, has a total bill of ₹43.11K on the Patient page, which includes ₹25,000 for surgery, ₹10,000 for rooms, ₹5,000 for tests, ₹2,000 for other expenses, ₹850 for fees, and ₹260 for medication. His mix reflects the overall trend, with surgical revenue also dominating at the individual level.

11. DOCTOR ANALYSIS – PROFILE & WORKLOAD

A 360-degree view of a physician's contribution is offered by the Doctor dashboard. Dr. Aditya Gupta, a cardiologist with an MD degree and a monthly salary of ₹300,000 who is currently listed as available, is the visible example. A patient billing table, appointment list, commission calculator, and commission gauge are all included on the page.

Field	Value	Interpretation
Name	Dr. Aditya Gupta	Sample doctor profile used for operational analysis
Specialization	Cardiologist	Matches the dominant 31-60 age-group patient segment
Qualification	MD	Indicates senior consultant-level expertise
Salary (Monthly)	₹3,00,000	Fixed operational cost independent of patient volume
Availability	Available	Doctor is currently open for appointments and cases
Rating	★★★★★	Indicates excellent patient satisfaction and service quality
Commission Earned	₹8.40K	Variable earnings generated through procedures and surgeries
Commission Rate	10%	Percentage of patient billing allocated as doctor commission
Patient Spend	₹84.02K	Total hospital revenue generated from treated patients
Patient Fees	₹1.65K	Revenue earned through direct consultation charges

11.1 Doctor Workload

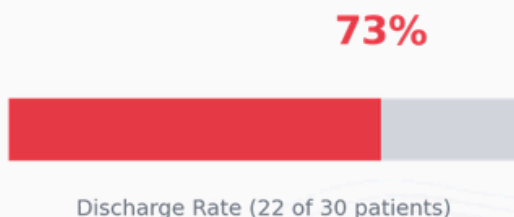
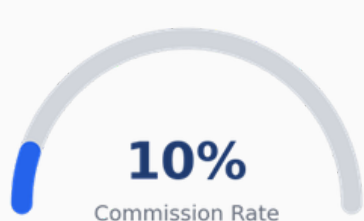
Two of Dr. Gupta's most recent appointments are listed on the Doctor page: Rahul Sharma on July 19 at 2:00 PM and October 25 at 2:00 PM. The Doctor's Appointment table at the hospital level displays several physicians along with their patients and the reasons for their visits (e.g., chest pain, follow-up check, heart palpitations, kidney pain, renal checkup, etc.). The predominant causes correspond to the cardiac and renal specialties that correspond to the age group specified in Section 8.

Capacity Finding:

The average number of patients per doctor on the panel, with 30 patients and 15 doctors, is 2. That is modest, even when different specialties are taken into account. If the hospital can change the mix toward surgery days, a larger patient-to-doctor ratio (e.g., 4–5) would significantly enhance the salary-to-revenue ratio without sacrificing care.

12. DOCTOR ANALYSIS – COMMISSION & EARNINGS

A commission calculator that allows the user to simulate various commission rates and patient-spend levels is available on the doctor site. In the default display, the sample doctor's commission earnings are ₹8.40K, with a commission rate of 10% on patient spending of ₹84.02K.



Doctor / Category	Patient Spend	Commission Rate	Commission Earned	Patient Fees
Dr. Aditya Gupta	₹84.02K	10%	₹8.40K	₹1.65K
Aggregate Hospital	₹713.81K Total Billing	—	₹71.38K	₹24.30K

Compensation Insight:

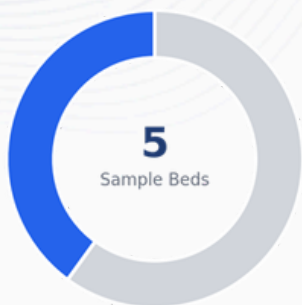
The hospital's total doctor commission is ₹71.38K, or roughly 10% of billings, which is in line with the sample doctor's individual rate. The sum of the doctor's costs (₹24.30K) and commission (₹71.38K) is ₹95.68K. Variable doctor remuneration is merely 2.3% of fixed costs, compared to a doctor's salary of ₹4.14 million. There is virtually little performance-linked compensation in the mix; the hospital pays doctors nearly exclusively on a salary basis.

Recommendation:

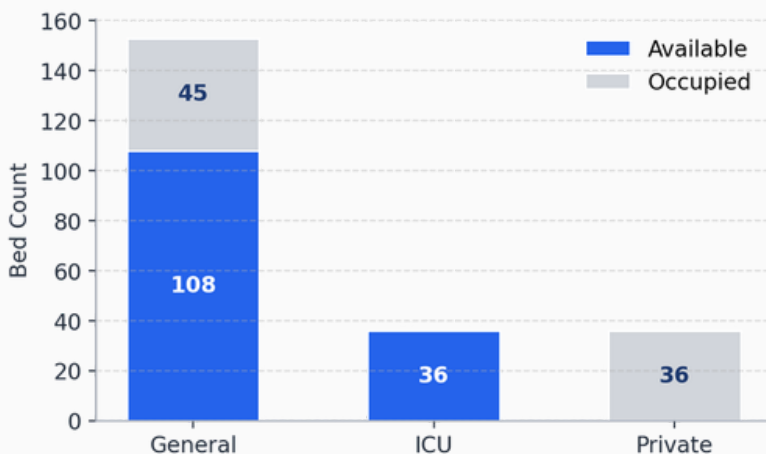
For senior experts, particularly in surgical fields, think about changing the compensation mix to include a larger commission component. Without significantly raising overall costs, even a shift from 2.3% to 5-7% variable pay would match incentives with revenue.

13. HOSPITAL OPERATIONS – BED STATUS

Two contrasting views of bed capacity are displayed on the dashboard. A little donut snapshot (2 available / 3 occupied) that depicts a filtered active view is displayed on the Overview page. The complete inventory, broken down by ward type, is displayed on the hospital page. For complete operational visibility, both are replicated below.



Overview snapshot (left) and hospital-wide occupancy (right).



Bed Status by Ward Type – Hospital page.

Ward Type	Available Beds	Occupied Beds	Total Beds	Occupancy %
General	108	45	153	29.4%
ICU	36	0	36	0.0%
Private	0	36	36	100.0%
Total	144	81	225	36.0%

Critical Finding:

The ICU is unoccupied, but the private ward is fully occupied. Out of all the signals on the dashboard, this one is the most important for operations. Either (a) demand has moved away from critical care and toward elective/private cases, or (b) ICU patients are being moved to private rooms after their condition stabilizes. In any case, the private ward, which generates more revenue, has a line while the ICU's capacity is idle.

14. HOSPITAL OPERATIONS – SURGERY & TESTS

Recent patient testing and planned surgeries are also listed on the hospital page. When taken as a whole, they explain the diagnostic process and clinical pathway.

14.1 Patient Tests (Hospital page snapshot)

Patient	Status	Result	Notes
Ajay Nair	Completed	Abnormal	Pulmonary fibrosis evident
Ajay Nair	Completed	Abnormal	Reduced lung capacity observed
Amit Kumar	Completed	Abnormal	Elevated liver enzymes identified
Amit Kumar	Completed	Abnormal	Gastric ulcers detected during examination
Amit Kumar	Completed	Normal	No parasites detected
Deepak Menon	Completed	Abnormal	Kidney stones visualized in scan

Patient	Status	Result	Notes
Deepak Menon	Completed	Abnormal	Reduced kidney function observed
Dinesh Agarwal	Completed	Abnormal	Cardiac irregularities noted during testing

Diagnostic Pattern:

Seven of the eight visible tests had abnormal results. According to the chronic-disease patient profile in the 31–60 age range, confirmed-pathology cases (pulmonary, hepatic, renal, cardiac, and gastric) account for the majority of the hospital's diagnostic effort.

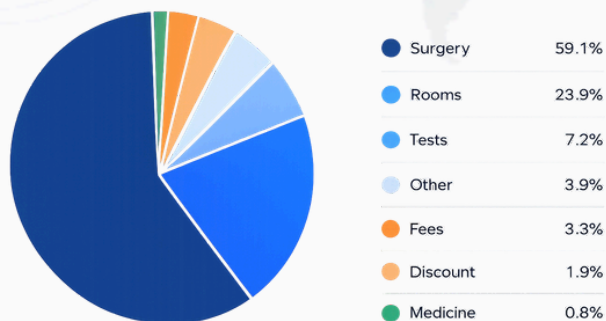
14.2 Surgery Schedule (Hospital page snapshot)

Patient	Appointment Time	Date
Rahul Sharma	10:00 AM	14-May
Priya Singh	11:30 AM	17-May

Pipeline Insight:

On the dashboard, every planned procedure is marked with a checkmark (✓). Every verified operation is materially valuable since surgery produces ₹430.00K, the greatest revenue line. The largest operational lever for revenue is keeping a careful eye on this pipeline.

15. FINANCE – REVENUE & CHARGES



Revenue concentration—Patient charges by category (% share).

15.1 Revenue Concentration

The concentration issue is evident when the Patient Charges Type chart is re-expressed as a share-of-total view: Surgery + Rooms = 84% of income. The hospital's revenue could drop by as much as 80% during the impacted period if either service line is interrupted (for example, a surgeon quits or the ICU closes for maintenance).

Category	Amount (₹K)	Share	Concentration Tier
Surgery	430.00	59.8%	Tier-1 (Dominant)
Rooms	174.00	24.2%	Tier-1 (Dominant)
Tests	52.00	7.2%	Tier-2 (Supporting)
Other	28.00	3.9%	Tier-3 (Auxiliary)
Fees	24.30	3.4%	Tier-3 (Auxiliary)

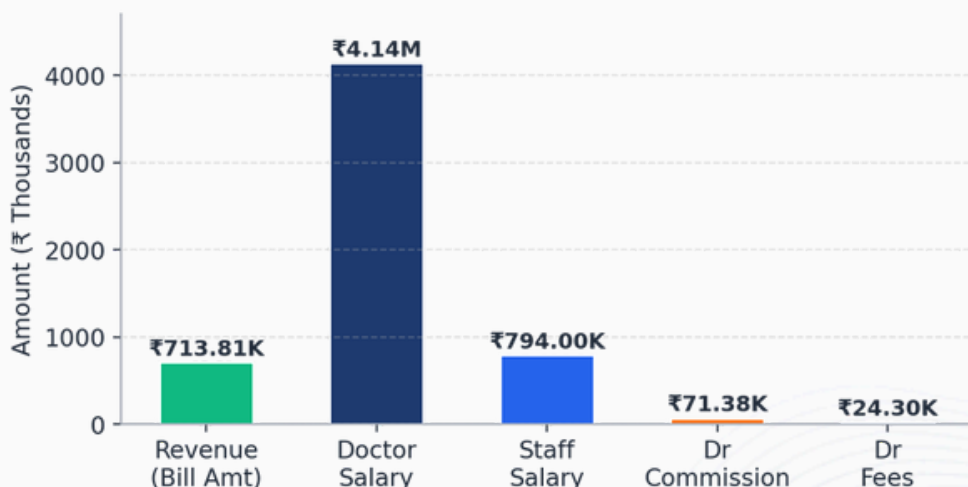
Category	Amount (₹K)	Share	Concentration Tier
Discount	13.30	1.9%	Negative (Concession)
Medicine	5.51	0.8%	Tier-3 (Auxiliary)

Risk Area:

Only when both service lines have steady, separate demand generators can revenue dependence on them be justified. Surgery is not divided between elective and emergency categories on the screen, which would influence resilience. In a later version of the dashboard, present this division.

16. FINANCE — EXPENSES & MARGIN

On its KPI grid, the Finance page displays three sizable expense lines: Doctor Salary (₹4.14M), Staff Salary (₹794K), and Doctor Commission (₹71.38K). One source of income is doctor fees (₹24.30K). The main monetary flows are contrasted side by side in the chart below.



Major monetary flows—revenue vs. core expense items (₹ thousands).

Item	Amount (₹)	Type
Bill Amount (Revenue)	₹713,810	Revenue
Doctor Salary	₹4,140,000	Fixed Expense
Staff Salary	₹794,000	Fixed Expense
Doctor Commission	₹71,380	Variable Expense
Doctor Fees	₹24,300	Revenue (Consultations)
Total Fixed + Variable Cost	₹5,005,380	Combined Operational Cost
Operating Position (Bill – Cost)	(₹4,291,570)	Net Deficit if Evaluated in Isolation

Observation:

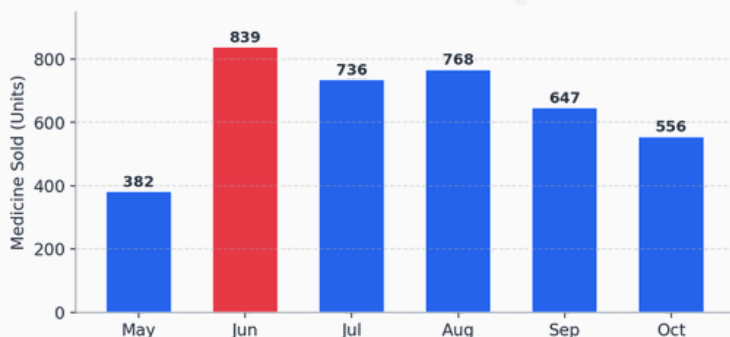
Only net patient billing is included in the ₹713.81K reported as the bill amount. The dashboard does not display the complete revenue from pharmacy retail (3,928 units × ₹158 = ₹620.6K), insurance settlements, or supplier rebates. As a result, the seeming shortfall mentioned above is not a real P&L deficit but rather an artifact of dashboard scope. A 'Total Revenue' KPI that compiles all sources of income would be beneficial for the dashboard.

Recommendation:

To the Finance page, add a single composite revenue KPI: Total Revenue = Bill Amount + Pharmacy Retail + Insurance Settlements. Leadership could overestimate profitability without it if they only looked at the dashboard.

17. FINANCE – MONTHLY TRENDS

The 'Monthly Med Sale' bar chart on the Finance page shows the number of medicine units sold each month from May to October of 2023. On the initial dashboard, June is indicated as the highest month.



Monthly Medicine Sale – peak in June (839 units).

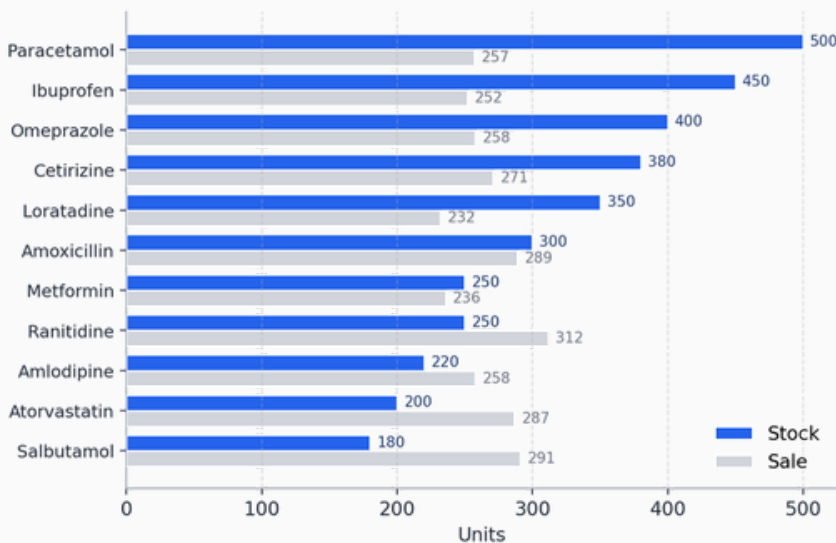
Month	Med Units Sold	MoM Change	Revenue (₹K, Estimated)*
May	382	—	60.3
Jun	839	+119.6%	132.6
Jul	736	-12.3%	116.3
Aug	768	+4.3%	121.3
Sep	647	-15.8%	102.2
Oct	556	-14.1%	87.8
Total	3,928	—	₹620.6K

Trend Analysis:

June is an anomaly that merits further research. A seasonal sickness outbreak, which is typical in monsoon-onset India; a one-time bulk dispensing to a corporate client; or a shift in the stocking cycle might all contribute to a 119.6% month-over-month increase in pharmaceutical sales. The next four months show a consistent decline, indicating either a one-time occurrence or the depletion of the June stock build-up. Verify the reason; recurring incidents influence the procurement for the following year.

18. MEDICINE & INVENTORY ANALYSIS

Medicine stock status is displayed on the finance page as a grouped horizontal bar chart that contrasts 11 medications' stock (blue) with sales (grey). The dashboard's most useful inventory view is this one.



Medicine Stock vs. Sales units.

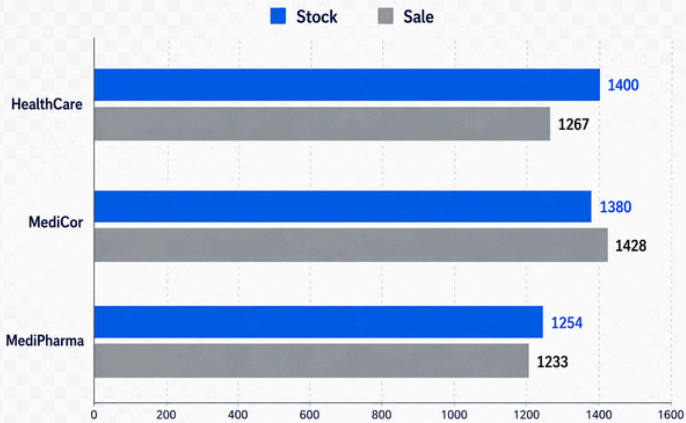
Medicine	Stock	Sale	Sale/Stock %	Status
Paracetamol	500	257	51.4%	Healthy
Ibuprofen	450	252	56.0%	Healthy
Omeprazole	400	258	64.5%	Healthy
Cetirizine	380	271	71.3%	Watch
Loratadine	350	232	66.3%	Healthy
Amoxicillin	300	289	96.3%	Critical
Metformin	250	236	94.4%	Critical
Ranitidine	250	312	124.8%	Stock-out
Amlodipine	220	258	117.3%	Stock-out
Atorvastatin	200	287	143.5%	Stock-out
Salbutamol	180	291	161.7%	Stock-out

Risk Area:

Four medications—salbutamol, amlodipine, atorvastatin, and ranitidine—have already sold more than they had on hand. Acid reflux, blood pressure, cholesterol, and asthma are all chronic care medications. Three of the four (salbutamol, amlodipine, and atorvastatin) directly benefit patients aged 31 to 60. Restock right away.

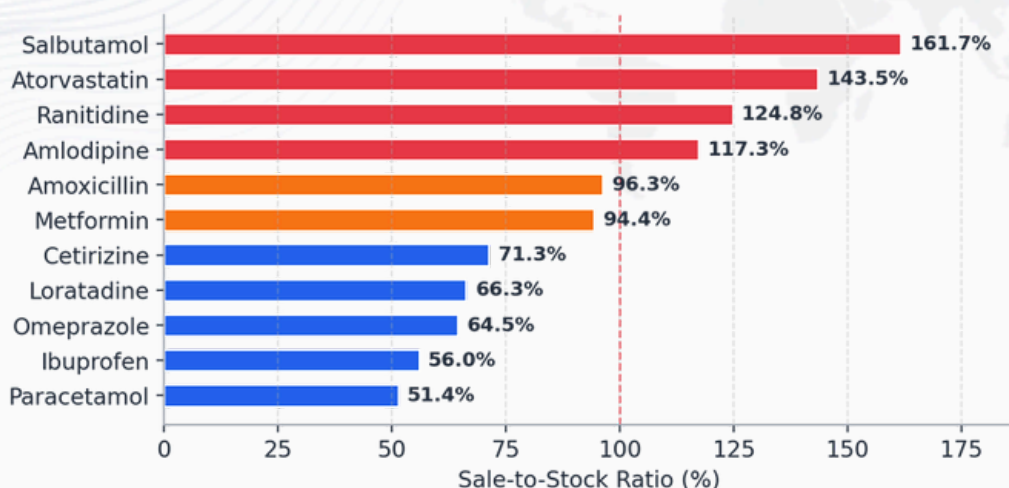
19. SUPPLIER STOCK ANALYSIS

Aggregate stock versus sales for the three suppliers that supply the hospital's pharmacy are displayed in the Supplier Stock Status graphic on the Finance page.



Supplier Stock vs. Sale — by vendor.

Supplier	Stock	Sale	Coverage	Status
HealthCare	1,400	1,267	110.5%	Adequate
MediCor	1,380	1,428	96.6%	Under-supplied
MediPharma	1,250	1,233	101.4%	Borderline
Total	4,030	3,928	102.6%	Hospital-wide Balance



Sale-to-Stock Ratio per medicine—bars above 100% indicate active stock-out risk.

Supplier Mix Insight:

The hospital's overall coverage is 102.6%, with supply (4,030) barely meeting demand (3,928). MediCor is already running low, though. The easiest short-term mitigation is to reallocate from HealthCare (over-supplied) to MediCor (under-supplied).

20. APPOINTMENTS & SCHEDULING

A list of forthcoming appointments (with the patient's photo, time, and date) appears on the Overview page. A more detailed doctor's appointment table with the doctor's name, patient's name, cause, and recommendation may be found on the hospital site. Additionally, the Patient page sample displays the context of each patient's appointment.

Doctor	Patient	Reason	Suggestion
Dr. Aditya Gupta	Rahul Sharma	Chest Pain	Prescribed medication
Dr. Aditya Gupta	Rahul Sharma	Follow-up Check	₹800 consultation advised
Dr. Aditya Gupta	Ravi Khanna	Heart Checkup	₹850 consultation completed
Dr. Aditya Gupta	Ravi Khanna	Heart Palpitations	ECG performed
Dr. Amit Khanna	Deepak Menon	Kidney Pain	Diagnostic tests ordered
Dr. Amit Khanna	Kishore Kumar	Renal Checkup	Hospital admission advised

Observation:

The causes for appointments fall into two categories: renal (kidney pain and renal checkup) and cardiac (chest pain, heart palpitations, heart checkup, and follow-up). Patients who come in today will show up in the admissions, surgery, and revenue figures tomorrow, according to the appointment table, which is a leading indicator.

Scheduling Insight:

Ajay Nair's times, such as 01:00 PM on August 4 and 01:00 PM on July 4, are displayed simultaneously on the Overview Appointments window. The hospital seems to concentrate appointments in the middle of the week, as evidenced by the medicine heatmap, which shows peak activity on Wednesdays (1,008 units total) and Tuesdays (849). To reduce the staff's weekly workload, think about distributing appointments.

21. KEY BUSINESS INSIGHTS

Ten conclusions jump out when all dashboard pages are combined. High-leverage discoveries are placed first, roughly according to business priority.

Insight

The main source of income is surgery. Surgery accounts for 60% of patient charges, or ₹430K out of ₹719K. When compared to consultation-only visits, the revenue is multiplied by approximately 6× for each subsequent surgery scheduled.

Insight

Demand for private wards exceeds that of intensive care units. The hospital should think about converting underutilized ICU capacity into additional private rooms (where safe) if private rooms are at 100% occupancy and the ICU is at 0%.

Insight

There is now a stock-out of four medications. Atorvastatin, salbutamol, ranitidine, and amlodipine have all sold more units than the available supply. The majority of patients are served by these chronic-care medications.

Insight

97.7% of doctor compensation is set. Just ₹95.68K (2.3%) of the total doctor payout (₹4.21M) is variable. For senior specialists, performance-linked compensation could be implemented.

Insight

The middle of the week is hectic. Sales of medications peak on Tuesday (849 units) and Wednesday (1,008 units from May to October). This is reinforced by appointment schedules. Stress and wait times are decreased by flattening the week.

Insight

There was a clinical and commercial surge in June. In June, the number of discharges peaked at five, while the number of medication units peaked at 839. Procurement and rostering must be 30 days ahead of schedule each year if this is a regular monsoon trend.

Insight

Drug mix is determined by demographics. The 31-60 band comprises 76.6% of patients. The medications that are running low are amlodipine, atorvastatin, and metformin, all of which are either approaching or below stock-out due to their chronic-disease burden.

Insight

The economics of pharmacies are excellent. With a 100% markup (₹158 sale vs. ₹79 cost), each extra unit sold generates ₹79 in gross profit. 3,928 units indicate around ₹310K in pharmacy GP, or nearly half of patient billings.

Insight

Although generally favorable, patient satisfaction is susceptible from an operational standpoint. A 5-star overview rating, but "had to wait too long" is mentioned in the patient sample's 3-star evaluation. Even when clinical care is evaluated well, wait times are a source of friction.

Insight

The dashboard scope has a serious cost-coverage gap. ₹713K in bills against the doctor + The ₹4.93 million staff wage demonstrates that the display does not fully depict revenue. It is imperative to include a total-revenue KPI.

22. RISK AREAS & OPPORTUNITIES

22.1 Risk Areas

Risk Area

Inventory shortages. Patients undergoing active treatment may miss doses if replenishment is delayed because four out of eleven medications are already sold out.

Risk Area

Revenue dependence on a single physician. The cardiac case pipeline that is displayed on the appointment table will stall if Dr. Gupta (Cardiology) is not available.

Risk Area

Two service lines account for the majority of revenue. Since surgery and rooms account for 84% of revenue, any disruption to surgical capability would have disastrous effects on revenue.

Risk Area

Overload in the middle of the week. Patient feedback has previously indicated that Tuesday and Wednesday peaks increase the risk of lengthier wait times and decreased patient satisfaction.

Risk Area

Scope gap in the dashboard. The total revenue amount is not full because insurance and pharmacy retail are not included in the bill amount, which gives the false impression of a deficit.

22.2 Opportunities

Opportunity

Convert the ICU's idle capacity. 36 intensive care unit beds are empty. Partial conversion to private or semi-private use unlocks bed-day revenue (~₹174K currently at 100% occupancy), subject to clinical safety.

Opportunity

Growth in pharmacy profits. 100% markup plus a patient base for chronic care. About ₹310K in pharmacy GP might be institutionalized through a subscription/repeat-prescription program for the five chronic-care medications.

Opportunity

Growth of surgical capacity. Every surgery makes a disproportionate financial contribution. Improving scheduling and ICU preparation to shorten the surgery lead time immediately raises the amount billed.

Opportunity

Bonus for doctor performance. It is anticipated that increasing variable doctor compensation from 2.3% to 5-7% will increase surgical throughput at little additional expense by aligning physician incentives with bookings.

Opportunity

Enhancement of the patient experience. The only thing that patients have complained about is wait times. It is inexpensive and has a significant impact to enhance queue visibility (digital tokens) and smooth the weekly load.

23. RECOMMENDATIONS – OPERATIONAL

Without any structural changes, the operations team can complete these tasks in 30 days. Every suggestion is based on a particular dashboard finding.

Recommendation

Set off an instant reorder for four medications that are out of stock. Atorvastatin, salbutamol, ranitidine, and amlodipine. Place orders within the week, giving MediCor priority because their portfolio is understocked.

Recommendation

Rebalance the mix of suppliers. Transfer incremental orders from HealthCare (110% coverage) to MediCor (96.6% coverage) until all three coverage levels stabilize at 100–105%.

Recommendation

Weekly scheduling is easy. Redistribute to Mondays and Thursdays and cap midweek appointment slots at 80% of the present peak. Reduction in wait-time complaints as shown by ratings is the anticipated result.

Recommendation

Include the ICU-to-private conversion process. Eight of the 36 ICU beds should be made available for short-term private use when it is clinically safe to do so. monetizes idle inventory while maintaining ICU surge capacity.

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Recommendation

A computerized queue display should be introduced. Respond to patient complaints that they "had to wait too long." Cost: little. Impact: evident in the subsequent cycle of enjoyment.

Recommendation

Create a KPI watchlist every week. Discharge %, Bill Amount per Patient, Stock-out Count, Type of Bed Occupancy, and June-style monthly surges. Every Monday, review.

24. RECOMMENDATIONS—STRATEGIC

Strategic Recommendation

Make doctors' salaries more changeable. For a quarter, test a 5% commission band for senior surgical doctors. Keep an eye on the effect on the revenue from the surgery line, which is now ₹430K. A 10–15% increase in the volume of elective surgeries is the hypothesis.

Strategic Recommendation

Start a subscription to a chronic-care pharmacy. Amlodipine, Atorvastatin, Metformin, Salbutamol, and Omeprazole are the five chronic-care medications. Provide a 5% loyalty discount on monthly refills. ensures recurring income from the 31–60 age group, which now makes up the majority of patients.

Strategic Recommendation

15% more surgeries should be performed. Surgery has the highest per-unit margin, accounting for 60% of revenue. Billing might reach ₹820K with a 15% increase in throughput. Examine whether the binding constraint is surgical schedule, anesthesia availability, or operating room (OT) hours.

Strategic Recommendation

On the Finance dashboard page, provide a Total Revenue KPI. Incorporate insurance settlements and pharmacy retail ($3,928 \times ₹158 \approx ₹620.6K$). For monthly evaluations, this provides leadership with a single, credible figure.

Strategic Recommendation

Create a dashboard display for the patient cohort. Divide patients according to the type of chronic illness (cardiac, renal, hepatic, or respiratory); the patient tests data already imply this. Outcome benchmarking and targeted campaigns are made possible by cohort tracking.

Strategic Recommendation

Arrange capacity for the June spike. Pre-staff and pre-stock 30 days in advance if June 2024 continues the 2023 pattern (839 medication units, 5 discharges). Instead than treating June as a surprise, treat it as a "demand peak" SOP.

25. Conclusion

A well-organized Power BI solution, the Hospital Dashboard effectively combines financial, clinical, and operational data into a single, user-friendly display. The hospital serviced 30 patients with a 73% discharge rate during the May–October 2023 period, made ₹713.81K in patient billings (of which ₹430K came from surgery alone), and moved 3,928 medication units at a healthy 100% markup.

The data's strongest indication is that the hospital's economics are driven by both the private ward and surgery, with the pharmacy being the most underappreciated profit center. The four medication stock-outs, which all cater to the middle-aged chronic-care patient group that predominates in intake, are the most critical operational signal. Strategically, the three levers most likely to significantly boost performance over the next two quarters are doctor compensation, surgical throughput, and dashboard scope (a missing Total Revenue KPI).

Each suggestion in Sections 23 and 24 is based on data that the dashboard has already collected. Adopting them just requires a disciplined operational cadence around the data on these pages, not additional technology. Instead of baselines, the subsequent iteration of this study can monitor changes.

30

Patients

₹713.81K

Billing

3,928

Med Units

73%

Discharge

Final Verdict:

Net Position: The hospital has strong operations and high demand. Pharmacy monetization, surgical throughput, and ICU/Private balance are its three main potential. Stock-outs, dashboard scope gaps, and reliance on single-line income are its three main dangers. Both lists can be used right now.